

1. A client with an indwelling urinary catheter reports lower abdominal pressure and only 50mL of urine in the bag over the past hour. What is the nurse's first action?

- a. Replace the catheter with a larger size
- b. Irrigate the catheter with 30mL normal saline
- c. Clamp the catheter for 15 minutes to build pressure
- d. Check the tubing for kinks and ensure the drainage bag is lower than the bladder

Answer: (d) Check the tubing for kinks and ensure the drainage bag is lower than the bladder

Explanation: The first step when a client with an indwelling urinary catheter has low urine output and discomfort is to check for mechanical obstructions. Kinks in the tubing or a drainage bag positioned higher than the bladder can impede gravity drainage. Other options are inappropriate as initial actions.

2. Which of the following listed instrument is used to hold and manipulate the uterus during gynecological procedures?

- a. Uterine sound
- b. Trocar

C. Retractor

- c. Sim's speculum

Answer: (a) Uterine sound

Explanation: A uterine sound is a gynecological instrument used to measure the depth and direction of the uterus. It is used to hold and manipulate the uterus during certain procedures. A trocar is an instrument used to access a cavity, a retractor holds back tissue, and a Sim's speculum is used to visualize the vagina and cervix.

3. As an advanced student nurse who is preparing for your licensure exams, how should the junior nurse ensure a proper fit of the rebreather mask?

- a. Use a larger-sized mask to ensure better oxygen delivery
- b. Secure the mask tightly to the patient's face
- c. Allow the patient to adjust the mask as desired
- d. Adjust the straps to ensure a snug fit without discomfort

Answer: (d) Adjust the straps to ensure a snug fit without discomfort

Explanation: A rebreather mask requires a snug fit to ensure adequate oxygen delivery and prevent air leakage. Proper adjustment of the straps achieves this without causing discomfort to the patient. Using a larger mask or allowing the patient to adjust it would compromise the seal, and securing it too tightly would cause discomfort.

4. To promote healing of a large incision, a patient should be given which of these vitamins?

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b. Amino acid

b. Ascorbic acid

c. Calcium

d. Thamine

Answer: (b) Ascorbic acid

Explanation: Ascorbic acid (Vitamin C) is crucial for collagen synthesis and wound healing, which is essential for the healing of a large incision. Amino acids are building blocks of protein, calcium is for bone health, and thiamine (Vitamin B1) is involved in metabolism.

5. Which of the following underlisted instrument is used to hold and manipulate tissues during delicate procedures?

a. Rongeur

b. Hemostat

c. Retractor

d. Trocar

Answer: (b) Hemostat

Explanation: A hemostat is a surgical instrument used to clamp blood vessels or hold and manipulate delicate tissues during procedures to control bleeding and provide

exposure. A rongeur is used for gnawing away bone, a retractor holds back tissues, and a trocar is used to access body cavities.

6. In the treatment and management of wounds, the acronym TIME is generally adopted. This TIME acronym in wound assessment stands for:

a. Temperature, Infection, Moisture, Edema

Tissue, Infection/inflammation, Medication, Elevation

b. Tissue, Inflammation/Infection, Moisture, Edge

c. Trauma, Infection, Medication, Edema

Answer: (c) Tissue, Inflammation/Infection, Moisture, Edge

Explanation: The TIME acronym is a framework used for systematic wound assessment and preparation for healing. Each letter represents a key component:

T – Tissue (non-viable or deficient)

I – Infection or Inflammation

M – Moisture imbalance

E – Edge of wound (non-advancing or undermined)

7. The nurse enters a client's room to assess the client, who began receiving a blood transfusion 45 minutes earlier, and notes that the client is flushed and dyspneic. The nurse determines that this client most likely is experiencing which complication of blood transfusion therapy?

a. Bacteriemia

b. Transfusion reaction

c. Hypovolemia.

d. Fluid overload

Answer: (b) Transfusion reaction

Explanation: The client's symptoms of being flushed and dyspneic (shortness of breath) occurring 45 minutes into a blood transfusion are classic signs of an acute transfusion reaction, which is an immune response to the transfused blood.

8. In Modern advanced clinical settings. Stem cell therapy in wound healing is promising because:

a. It prevents all infections yet prognosis is poor

b. It is the only option

c. It is inexpensive hence not appropriate for the poor

d. It promotes angiogenesis and tissue regeneration

Answer: (d) It promotes angiogenesis and tissue regeneration

Explanation: Stem cell therapy is promising in modern clinical settings because stem cells have the ability to differentiate into various cell types and secrete growth factors, which promotes the formation of new blood vessels (angiogenesis) and the regeneration of damaged tissue.

9. Mr. Kyle with O+ blood is in need of an emergency transfusion in a tertiary Healthcare facility but the lab does not have any O+ blood available. Which potential unit of blood could be given to the client?

- a. B+ unit
- b. Any Rh+ unit
- c. O-unit
- d. A+ unit

Answer: (c) O- unit

Explanation: Individuals with O- blood type are considered universal donors because their red blood cells lack A, B, and Rh antigens, meaning they can be safely transfused to patients of any blood type in emergency situations when cross-matching is not feasible.

9. The reaction or response of the body tissue to irritation or injury is termed:

- a. Infection
- b. Resolution
- c. Suppuration

e. Inflammation

Answer: (d) Inflammation

Explanation: Inflammation is the body's natural physiological response by vascularized tissues to harmful stimuli, such as pathogens, damaged cells, or irritants. It involves a series of cellular and chemical responses leading to redness, swelling, heat, pain, and loss of function at the site of injury or irritation.

10. A patient with O blood received A+ blood. The patient is at risk for?

- a. Febrile transfusion reaction
- b. Allergic transfusion reaction
- c. None: O+ and A+ are compatible blood types
- d. Hemolytic transfusion reaction

Answer: (d) Hemolytic transfusion reaction

Explanation: A patient with O blood type has anti-A and anti-B antibodies in their plasma. If they receive A+ blood, these antibodies will attack the A antigens on the transfused red blood cells, leading to a severe and potentially fatal acute hemolytic transfusion reaction.

11. Which of the following should the nurse assess first when a patient is using a rebreather mask for oxygen therapy?

- a. Respiratory rate and effort
- b. Blood pressure and heart rate
- c. Skin integrity around the mask area
- d. Oxygen flow rate and temperature

Answer: (a) Respiratory rate and effort

Explanation: When a patient is using a rebreather mask, the primary concern is adequate oxygenation and ventilation. Assessing the respiratory rate and effort first ensures the patient is breathing effectively and that the mask is providing sufficient support for their immediate respiratory needs.

12. Which action by the nurse would best reduce the risk of catheter-associated urinary tract infection (CAUTI)?

- a. Maintain a closed drainage system and avoid unnecessary disconnections
- b. Change the drainage bag daily
- c. Disconnect the catheter from the bag once per shift for cleaning
- d. Flush the catheter daily to prevent sediment buildup

Answer: (a) Maintain a closed drainage system and avoid unnecessary disconnections

Explanation: The most effective way to prevent CAUTI is to maintain the sterility of the urinary system. Keeping the drainage system closed prevents the entry of bacteria from the external environment into the sterile urinary tract.

13. Inflammation is a useful response which serves the following purposes EXCEPT

- a. Localize and eliminate the causative agent
- b. Respond to a particular cause
- c. Restore tissue to normality or near normal
- d. Limit tissue injury

Answer: (b) Respond to a particular cause

Explanation: Inflammation is a non-specific response to injury or infection. It does not respond to a particular cause in a targeted manner like the adaptive immune system; rather, it is a general, immediate reaction to a variety of stimuli. Its purposes include localizing the agent, limiting injury, and restoring tissue to normality.

14. Before moving a client up in bed, the nurse lowers the head of the bed. The purpose of lowering the head of the bed is

- a. To prevent getting wrinkles in the client's linen
- b. To relieve pressure on the client's sacrum

- c. To avoid working against gravity as the client is moved
- d. To eliminate needing additional help to move the client

Answer: (c) To avoid working against gravity as the client is moved

Explanation: Lowering the head of the bed makes the client's body parallel to the bed surface. This position reduces the effect of gravity, making it physically easier for the nurse to move the client up in the bed and reducing the risk of injury to both the client and the nurse.

15. Immediate post-operative hazards include the following EXCEPT?

- a. Vomiting
- b. Haemorrhage
- c. Sepsis
- d. Shock

Answer: a. Vomiting

Vomiting is not considered an immediate post-operative hazard in the same category as the others listed. Haemorrhage, sepsis, and shock are major, life-threatening complications that require immediate medical intervention post-surgery. While vomiting can occur after surgery, it is typically a less severe and more manageable side effect (often related to anesthesia), unlike the systemic and critical risks posed by the other options.

16. Which statement is the rationale for infusing a unit of blood in less than four (4) hours?

- a. The blood components begin to break down after four (4) hours.
- b. The blood will coagulate if left out of the refrigerator for >four (4) hours.
- c. The blood will not be affected; this is a laboratory procedure.
- d. The blood has the potential for bacterial growth if allowed to infuse longer.

Answer: d. The blood has the potential for bacterial growth if allowed to infuse longer.

The primary reason for completing a blood transfusion within four hours is to minimize the risk of bacterial proliferation. Blood products are highly susceptible to bacterial contamination once they are removed from controlled refrigeration, and bacteria can multiply to dangerous levels if the infusion time is extended beyond this limit.

17. When assessing a lesion diagnosed as malignant melanoma, the nurse in charge most likely expects to note which of the following?

- a. A pearly papule with a central crater and a waxy border
- b. A small papule with a dry, rough scale
- c. An irregular shaped lesion

- d. A firm, nodular lesion topped with crust

Answer: c. An irregular shaped lesion

Malignant melanomas are typically characterized by asymmetry, irregular borders, variations in color, a diameter greater than 6mm, and evolution over time (the ABCDE criteria). An irregular shape is a key diagnostic feature used by healthcare professionals to identify potential melanoma.

18. A gravida II para O is admitted to the labor and delivery unit. The doctor performs an amniotomy. Which observation would the nurse expect to make after the amniotomy?

- a. A small amount of greenish fluid
- b. A small segment of the umbilical cord
- c. Fetal heart tones 160bpm
- d. A moderate amount of straw-colored fluid

Answer: d. A moderate amount of straw-colored fluid

A normal finding after an amniotomy (artificial rupture of membranes) is the release of amniotic fluid that is clear to straw-colored and moderate in amount. Greenish fluid indicates the presence of meconium, which can suggest fetal distress, while the presence of the umbilical cord is an emergency (prolapsed cord). Fetal heart tones of 160 bpm are slightly elevated, but the expected fluid characteristic is the key observation here.

19. Inflammation is a useful response which serves the following purposes EXCEPT

- a. Restore tissue to normality or near normal
- b. Localize and eliminate the causative agent
- c. Respond to a particular cause
- d. Limit tissue injury

Answer: c. Respond to a particular cause

Inflammation is a generalized, non-specific response to injury or infection. It does not “respond to a particular cause” in a specific, targeted manner like the adaptive immune system does. Instead, its general purposes include localizing the agent, limiting injury, and initiating tissue repair.

20. What is the priority nursing action when caring for a patient with antepartum hemonhage?

A Assisting with vaginal exams

Providing emotional support

c. Administering pain medication

d. Monitoring fetal heart rate

Answer: (d) Monitoring fetal heart rate Monitoring the fetal heart rate is the priority nursing action because antepartum hemorrhage can lead to fetal distress, and the well-being of both the mother and the fetus must be assessed and managed immediately.

21. Nurse Awusiwas was dressing a wound on the surgical unit and observed that exudates from the wound appear clear and watery. She documents this as

- a. Sero-sanguinous
- b. Sanguinous
- c. Purulent
- d. Serous

Answer: (d) Serous

Serous exudate is a clear, watery plasma, which is a normal finding during the early stages of wound healing. Sanguinous is bloody, sero-sanguinous is pinkish, and purulent is thick and often yellow, green, or brown, indicating infection.

23. Which of the following is a characteristic of an unintentional wound is

- a. Clean edges
- b. Done on purpose
- c. Bleeding is controlled

- d. High risk for infection

Answer: (d) High risk for infection

Unintentional wounds, such as those from accidental trauma, typically have jagged edges and are exposed to non-sterile environments, leading to a high risk for infection. Intentional wounds (like surgical incisions) have clean edges and controlled bleeding, and are performed under sterile conditions.

24. The nurse is caring for a client with ascites. Which is the best method to use for determining early ascites?

- a. Assessment for a fluid wave
- b. Inspection of the abdomen for enlargement
- c. Bimanual palpation for hepatomegaly
- d. Daily measurement of abdominal girth

Answer: (d) Daily measurement of abdominal girth

Daily measurement of abdominal girth is the most reliable and consistent method for determining early or gradual accumulation of ascites (fluid in the abdominal cavity). A fluid wave is a less sensitive indicator and is typically used when ascites is more advanced.

25. Which of the following is the most appropriate initial nursing action before initiating a blood transfusion?

- a. Checking the patient's identification
- b. Verifying the blood type and crossmatch
- c. Assessing the patient's vital signs
- d. Obtaining informed consent from the patient

Answer: (a) Checking the patient's identification

The most appropriate initial nursing action is checking the patient's identification against the blood product label with another nurse to ensure the correct blood is given to the correct patient, a critical safety measure. Verifying blood type and crossmatch is done in the lab, assessing vital signs comes next, and consent is typically obtained by a physician before the process begins.

26. Which of the following underlisted instrument is commonly used to remove sutures or staples from surgical incisions?

A Hemostat

b. Retractor

c. Suture scissors

e. Trocar

Answer: (c) Suture scissors

Suture scissors are specifically designed with a small hook on one blade to slide under the suture for removal, or for cutting staples. Hemostats are used to clamp blood vessels, retractors hold back tissue during surgery, and trocars are used to create an opening into a body cavity, not for removing sutures or staples.

27. A client is being catheterized for acute urinary retention. After the catheter drains and the bladder is emptied, the nurse should:

- a. Monitor output and consider clamping the catheter for 30 minutes
- b. Ensure free drainage and monitor for post void residuals or bladder spasms
- c. Inflate the balloon immediately and disconnect the bag
- d. Leave the catheter in place until the client asks to have it removed

Answer: (b) Ensure free drainage and monitor for post void residuals or bladder spasms

After the bladder is emptied, the primary concern is to ensure continuous, free drainage and monitor the client for potential complications like post-void residuals or bladder spasms. Clamping the catheter is generally not recommended as it can increase bladder pressure and potentially cause trauma. The balloon should be inflated as per protocol, and the catheter is left in place as ordered by the physician, not until the client asks for removal.

28. A client with a newly placed indwelling urinary catheter asks why the catheter is secured to the thigh. The nurse's best response is:

- a. "It keeps the catheter off the bed so it won't get dirty"

- b. "It helps prevent the tubing from pulling on your urethra and causing trauma."
- c. "It allows you to walk without dragging the bag behind you."
- d. "It's so you cannot move and accidentally pull the catheter out."

Answer: (b) "It helps prevent the tubing from pulling on your urethra and causing trauma."

Securing the catheter to the thigh (for females) or abdomen (for males) is crucial to prevent accidental pulling on the urethra, which can cause significant tissue trauma and pain. The other options are either incorrect or secondary benefits.

29. A client is receiving external radiation for cancer of the larynx. As a result of the treatment, the client will most likely complain of:

- a. Dyspnea
- b. Sore throat
- c. Bone pain
- d. Generalized pruritis

Answer: (b) Sore throat

Radiation therapy to the larynx often causes mucositis (inflammation and ulceration of the mucous membranes) in the throat, leading to a sore throat or difficulty swallowing (dysphagia). Dyspnea (difficulty breathing) is less likely unless there is significant swelling, bone pain is not a direct result of localized radiation to the larynx, and generalized pruritis (itching) is not a common side effect of this localized treatment.

30. A client's indwelling catheter has been in place for 10 days. According to best practice guidelines, the nurse's NEXT action should be

- a. Change the catheter every 7 days automatically
- b. Schedule a routine catheter change today to avoid infection
- c. Leave it in unless there is a specific indication to change or retrieve
- d. Remove the catheter and insert a new one immediately

Answer: (c) Leave it in unless there is a specific indication to change or retrieve

Best practice guidelines state that indwelling catheters should be left in place unless there is a specific clinical indication for removal or change, to minimize the risk of infection and trauma. Routine, time-based changes without indication are not recommended.

31. Which of the following is not a common visible cause of antepartum hemorrhage?

- a. Ectopic pregnancy

B Placenta previa

- b. Placental abruption
- c. Uterine rupture

Answer: (a) Ectopic pregnancy

An ectopic pregnancy occurs early in pregnancy, typically before the fetus is viable, and is not considered a common visible cause of antepartum hemorrhage (bleeding after fetal viability, typically after 20 weeks of gestation). Placenta previa, placental abruption, and uterine rupture are common causes of visible antepartum hemorrhage.

32. Which of the following is the CORRECT TECHNIQUE for obtaining a urine specimen from an indwelling urinary catheter?

- a. Irrigate the catheter with saline and collect the irrigant as specimen
- b. Disconnect the catheter from the bag, let urine collect in the bag, then withdraw from bag
- c. Use the drainage bag directly for culture because it contains fresh urine
- d. Clean the specimen port with antiseptic, clamp the tubing temporarily below the port, withdraw urine from port using a sterile syringe

Answer: (d) Clean the specimen port with antiseptic, clamp the tubing temporarily below the port, withdraw urine from port using a sterile syringe

This technique ensures a sterile, fresh urine sample is collected directly from the catheter system, minimizing contamination risks. Using the drainage bag or the irrigant is not an appropriate method for obtaining a sterile culture specimen.

33. The nurse is suspected of charting the administration of a medication that he did not give. After talking to the nurse, the charge nurse should:

- a. File a formal reprimand
- b. Charge the nurse with malpractice
- c. Charge the nurse with a tort
- d. Call the Board of Nursing to complain

Answer: (a) File a formal reprimand

Falsifying medical records is a serious professional misconduct issue. The charge nurse should follow the proper institutional protocol, which typically begins with a formal reprimand and may escalate to reporting to the Board of Nursing, depending on the severity and frequency. Malpractice and tort are legal terms that would be handled in a court of law, not by a charge nurse.

34. Honey is used in wound care primarily for its

- a. Color and texture properties
- b. Sweetness and bacterial properties
- c. Microbial properties
- d. Antimicrobial and healing properties

Answer: (d) Antimicrobial and healing properties.

Medical-grade honey is used in wound care for its significant antimicrobial properties, which help prevent and treat infection, and its ability to promote a moist wound healing environment.

35. Removal of devitalized tissue in pressure ulcers when appropriate for the resident's condition and consistent with resident goals is

- a. Debridement
- b. Sterile technique
- c. Irrigation
- d. Exudates

Answer: (a) Debridement

Debridement is the medical removal of dead, damaged, or infected tissue to improve the healing potential of the remaining healthy tissue.

36. A nurse is obtaining a urine specimen from an indwelling catheter. Which technique is correct?

- a. Disconnect the catheter from the drainage bag, allow new urine to collect in the bag, then extract the sample
- b. Clean the port with antiseptic, attach a syringe, and withdraw urine from the catheter port

- c. Clamp the catheter for 15 minutes, then collect urine from the bag
- d. Pull the catheter back slightly so fresh urine enters tubing, then collect from the tubing

Answer: (b) Clean the port with antiseptic, attach a syringe, and withdraw urine from the catheter port

To obtain a sterile or clean urine specimen from an indwelling catheter, the catheter tubing must be clamped below the sample port for about 15-30 minutes to allow fresh urine to collect in the tubing. The port is then cleaned with an antiseptic, a syringe is attached, and the urine is withdrawn. Collecting from the drainage bag or disconnecting the system increases the risk of infection.

37. Most peptic ulcers occurring in the stomach are in the

- a. Pyloric portion
- b. Cardiac portion
- c. Body of the stomach
- d. Oesophageal junction

Answer: (a) Pyloric portion

Most peptic ulcers in the stomach occur in the pyloric portion, which is the area connecting to the small intestine.

38. What is the appropriate nursing intervention for a patient with suspected placenta previa and is actively bleeding?

Preparing for an immediate cesarean section

bApplying pressure to the umbilical cord

d. Encouraging ambulation to promote blood flow

e. Administering oxytocin to stimulate uterine contractions

Answer: (a) Preparing for an immediate cesarean section

For a patient with suspected placenta previa who is actively bleeding, the primary nursing intervention is to prepare for an immediate cesarean section because the condition can cause severe hemorrhage and poses a significant risk to both the mother and the baby.

39. The nurse is taking the blood pressure of an obese client. If the blood pressure cuff is too small, the results will be:

a. A subnormal finding

b. A false low reading

c. A false elevation

d. A blood pressure reading that is correct

Answer: (c) A false elevation

A blood pressure cuff that is too small for a client, especially an obese client, will result in a reading that is artificially high or a false elevation. The cuff cannot properly compress the artery, leading to an overestimation of the pressure.

40. Which of the following is the most common cause of postpartum hemorrhage?

- a. Uterine atony
- b. Retained placenta
- c. Placental abruption
- d. Uterine rupture

Answer: (a) Uterine atony

Uterine atony, the failure of the uterus to contract sufficiently after childbirth, is the primary and most frequent cause of postpartum hemorrhage. The lack of contraction prevents the sealing of blood vessels at the site where the placenta was attached.

41. When planning to remove a client's indwelling urinary catheter, which nursing action is essential immediately BEFORE withdrawal?

- a. Pull the catheter gently while asking the client to breathe deeply

- b. Ask the client to void and then remove the catheter
- c. Cut the catheter tubing near the insertion site and allow it to fall out
- d. Deflate the balloon fully using a syringe and prepare for removal

Answer: (d) Deflate the balloon fully using a syringe and prepare for removal

It is critical to fully deflate the balloon within the bladder using a syringe before attempting to withdraw the catheter. Failure to do so would cause significant injury to the urethra and bladder neck.

42. Which action by the healthcare worker indicates a need for further teaching?

- a. The nurse wears gloves to take the client's vital signs.
- b. The nurse wears goggles while drawing blood from the client.
- c. The doctor washes his hands before examining the client.
- d. The nursing assistant wears gloves while giving the client a bath.

Answer: (a) The nurse wears gloves to take the client's vital signs.

Taking routine vital signs, such as blood pressure, pulse, and temperature, does not typically require the use of gloves, provided there is no anticipated contact with bodily fluids. Standard precautions like hand hygiene before and after are sufficient. The other actions (wearing goggles for blood draws, handwashing before examination, and wearing gloves for bathing) are appropriate infection control practices.

43. The nurse is teaching a client who will soon have an indwelling catheter removed. Which statement by the client indicates CORRECT UNDERSTANDING of what to expect post removal?

- a. "It's okay if I don't void for 24 hours after removal."
- b. "Once the catheter is out, I won't need to worry about residual urine"
- c. I might feel a strong urge to void and need to go right away."
- c. "I should avoid drinking too much fluid to prevent urgency."

Answer: (c) "I might feel a strong urge to void and need to go right away."

After the removal of an indwelling catheter, it is common for clients to experience bladder spasms or a strong, immediate urge to urinate as the bladder readjusts to normal function. It is important to inform clients of this possibility

44. Which of the following underlisted instruments is used to close surgical incisions or wounds?

- a. Stitch Scissors
- b. Trocar
- c. Suture material
- d. Needle holder

Answer: c. Suture material

Suture material (sutures) is used to close surgical incisions or wounds by holding the edges of the wound together to promote healing.

45. For a female client, the nurse begins insertion of a urinary catheter. How far should the catheter be advanced once urine begins to flow?

- a. 6-8 inches after urine appears
- b. 1 inch further past urine flow
- c. 2-3 inches then stop
- d. Until the client reports pain

Answer: b. 1 inch further past urine flow

Once urine flow is established, the catheter is advanced a little further (typically 1-2 inches) to ensure the balloon port is fully within the bladder before inflation.

46. Wound healing which has the margins neatly approximated is known as

- a. Regeneration

- b. Secondary intention
- c. Primary intention
- d. Tertiary intention

Answer: c. Primary intention

Wound healing by primary intention occurs when the edges of a clean wound are neatly approximated (brought together), leading to faster healing and minimal scarring.

47. Which of the following underlisted instrument is used to hold and manipulate needles during suturing?

- a. Suture material
- b. Needle holder
- c. Trocar
- d. Stitch Scissors

Answer: b. Needle holder

A needle holder (or needle driver) is a surgical instrument used by doctors and nurses to grasp and manipulate a suture needle during suturing.

48. Which of the following is not a cause of inflammation?

- a. Allergy
- b. Injury
- c. Leucocytosis
- d. Infection

Answer: c. Leucocytosis

Leucocytosis is an increase in the number of white blood cells, which is a result of the body's response to inflammation or infection, not a cause of inflammation itself. Allergy, injury, and infection can all trigger an inflammatory response.

49. The skin's main function in wound healing is:

- a. Vitamin D synthesis only
- b. Temperature regulation only
- c. Barrier protection and tissue repair
- d. Perspiration and growth of hair

Answer: (c) Barrier protection and tissue repair

The skin serves as a crucial protective barrier against external pathogens and environmental factors. In the context of wound healing, its primary functions involve restoring this barrier and facilitating the complex process of tissue repair through hemostasis, inflammation, proliferation, and remodeling.

50. Which of the following underlisted instrument is used to hold back tissues or organs to provide better visualization during vaginal examinations?

b. Forceps

d. Hemostat

e. Cusco

Answer: (d) Cusco

A Cusco's speculum is an instrument designed to be inserted into the vagina to hold back the walls of the vaginal canal, allowing for better visualization of the cervix and vaginal tissues during an examination or procedure.

51. You're assessing the stoma of a patient with a healthy, well-bealed colostomy. You expect the stoma to appear:

a. Dry and black

b. Dark or purple-colored

c. Pale, pink and moist

- d. Red and moist

Answer: (d) Red and moist

A healthy stoma should appear red or pink, similar in color to the mucous membrane lining the inside of the mouth, and moist. This indicates adequate blood flow and viability. Any other color (black, purple, or pale) would suggest a potential problem.

52. What are the advantages to using monofilament suture material?

- a. High Risk of infection, High Risk of Reactivity
- b. Strong Firm Knot, Minimal Tissue Reactivity
- c. Low Risk of infection, Is an Organic Product
- d. Minimal Tissue Reactivity, Reduced Risk of Infection

Answer: (d) Minimal Tissue Reactivity, Reduced Risk of Infection

Monofilament sutures are made of a single strand, which results in less tissue drag and minimal tissue reactivity compared to braided sutures. The smooth surface also means bacteria are less likely to adhere, reducing the risk of infection.

53. Midwife Serwaa understands that, in the process of blood transfusion and compatibility crisis, the most appropriate nursing action if a transfusion reaction is suspected will be to?

- a. Administer antihistamines and continue the transfusion

- b. Document the reaction and continue monitoring the patient
- c. Stop the transfusion immediately and notify the healthcare provider
- d. Increase the transfusion rate to flush out the reaction

Answer: (c) Stop the transfusion immediately and notify the healthcare provider

The first and most critical nursing action when a transfusion reaction is suspected is to immediately stop the transfusion to prevent further harm to the patient. The healthcare provider must then be notified to assess the patient and determine the appropriate course of action.

54. The nurse is caring for a 10-year-old client scheduled for surgery. The client's mother tells the nurse that her religion forbids blood transfusions. What nursing action is most appropriate?

A Explain the consequences of no treatment

b. Document the mother's statement in the chart

Encourage the mother to reconsider

- e. Notify the physician of the mother's refusal

Answer: (d) Notify the physician of the mother's refusal

In cases where a parent or legal guardian refuses a life-saving treatment like a blood transfusion for their child based on religious beliefs, the nurse's primary role is to ensure all relevant medical information is communicated to the appropriate personnel.

The physician then has the responsibility to explore alternatives, discuss the situation further with the family, and, if necessary, involve legal or ethical teams to determine the best course of action, which may include seeking a court order to override the refusal in the child's best interest.

55. Wound drainage is inserted in or near a wound to.....and decreases the risk of formation.....drainage,

- a. Discourage, abscess
- b. Promote, abscess
- c. Promote, tissue
- d. Cease, abscess

Answer: (b) Promote, abscess

Wound drainage is inserted in or near a wound to promote drainage and decreases the risk of abscess formation

56. A 45-year-old client returned from a colon resection 2 hours ago. Which vital signs indicate possible hemorrhagic shock?

- a. BP 160/98, heart rate 54
- b. BP 120/80, heart rate 88
- c. BP 170/100, heart rate 120

D BP 96/60, heart rate 120

Answer: (d) BP 96/60, heart rate 120

Hemorrhagic shock is characterized by significant blood loss, leading to decreased blood pressure (hypotension) and increased heart rate (tachycardia) as the body attempts to compensate for the reduced blood volume. Option (d) presents the classic signs of low blood pressure and a high heart rate, consistent with shock.

57. nurse is assessing a client after a car accident. Partial airway obstruction is suspected. Which clinical manifestation is a late sign of airway obstruction?

A Restlessness

b. Inspiratory stridor

Rales auscultated in breath sounds

d. Cyanotic ear lobes

Answer: (d) Cyanotic ear lobes

Cyanosis is a late sign of airway obstruction because it indicates that the body's tissues are not receiving adequate oxygen, a critical and delayed symptom of severe hypoxia. Restlessness and inspiratory stridor are typically early signs as the body attempts to compensate for the lack of air. Rales are crackling sounds in the lungs and are not directly indicative of an airway obstruction.

58. Which of the following practices is LEAST LIKELY to help reduce catheter-associated urinary tract infection (CAUTI) risk?

- a. Routinely changing drainage bags at fixed time intervals (eg. Every 7 days)
- b. Using the smallest sized catheter consistent with good drainage
- c. Maintaining a closed sterile drainage system
- d. Assessing daily whether the catheter is still needed and removing when possible

Answer: (a) Routinely changing drainage bags at fixed time intervals (e.g. every 7 days)

Routinely changing drainage bags at fixed time intervals is least likely to help reduce the risk of a catheter-associated urinary tract infection (CAUTI) because maintaining a closed sterile drainage system is a critical preventative measure. Opening the system unnecessarily for routine bag changes increases the risk of contamination. The other options—using the smallest possible catheter, maintaining a closed system, and daily assessment for removal—are all recommended practices for CAUTI prevention

59. Which of the following surgical consent would NOT be considered legal?

- a. Consent signed by a 60-year-old man the evening prior to surgery
- b. Consent signed by 21-year-old man who has a fractured left tibia due to an auto accident
- c. Consent signed by a 33-year-old manic man
- d. Consent signed by a 36-year-old lady one hour after receiving her preoperative medication

Answer: d. Consent signed by a 36-year-old lady one hour after receiving her preoperative medication

Explanation: A patient must be mentally competent and fully informed to give legal consent. Preoperative medications (such as sedatives or analgesics) can impair a patient's judgment and capacity to give informed consent, making it legally invalid. The other options describe situations where consent would generally be considered valid, assuming competence at the time of signing.

60. The patient admitted to the unit. Vaginal exam reveals that she is 3cm dilated. Which of the following statements would the nurse expect her to make?

- a. "When can I get my epidural?"
- b. "Don't touch me. I'm trying to concentrate."
- c. "I can't decide what to name the baby."
- d. "It feels good to push with each contraction."

Answer: a. "When can I get my epidural?"

Explanation: At 3 cm dilation (early active labor), a patient is likely experiencing increasing discomfort and pain. It is common for patients in this stage to inquire about pain relief options, such as an epidural. The other statements are less typical for this specific stage of labor: concentrating silently is more common in active labor, deciding on a name can happen at any time, and the urge to push usually occurs much later in labor (around 10 cm dilation).

61. Nurse Rosemond understands that observation and monitoring are paramount in blood transfusion. How long should it take to administer a unit of packed red blood cells during a blood transfusion?

- a. 1 hour
- b. 6 hours
- c. 2 hours
- d. 4 hours

Answer: d. 4 hours

Explanation: A unit of packed red blood cells (PRBCs) should be infused within 4 hours to minimize the risk of bacterial proliferation if the unit is contaminated. Infusing faster than 4 hours may be possible in some emergency situations, but 4 hours is the maximum safe duration for a standard transfusion.

62. Nurse Beatrice dressed a wound on the medical ward and observed that exudates from the wound appear light pink and blood tinged. She documents this as:

- a. Purulent
- b. Sanguinous
- c. Sero-sanguinous
- d. Serous

Answer: c. Sero-sanguinous

Explanation: Wound exudates are classified by their appearance.

Serous: Clear, watery plasma.

Sanguinous: Fresh red blood (bloody).

Purulent: Thick, yellow, green, or brown fluid (pus), often indicating infection.

Sero-sanguinous: Thin, watery, pale red to pink, blood-tinged plasma. This matches the description in the question.

63. During insertion of a urinary catheter in a male client, urine appears in the tubing
How far should the nurse advance the catheter before inflating the balloon?

a. Advance approximately 1-2 inches (2.55cm) past urine flow

b. Hold insertion where urine appeared and inflate

c. Advance 6-8 inches after urine appears

e. Immediately inflate the balloon upon seeing urine

Answer: a. Advance approximately 1-2 inches (2.5-5cm) past urine flow

Explanation:

When inserting an indwelling urinary catheter in a male client, once urine flow is observed, the catheter is advanced an additional 1 to 2 inches (2.5 to 5 cm) to ensure the balloon is fully within the bladder and not in the urethra, which could cause injury upon inflation.

64. The nurse is caring for a client following a right nephrolithotomy. Post-operatively, the client should be positioned

a. Prone

b. Supine

c. On the left side

Od. On the right side

Answer: c. On the left side

Explanation:

Following a nephrolithotomy (surgical incision into the kidney to remove stones), the client should be positioned on the operative side (the right side in this case) to promote drainage. However, since “on the right side” is not an option, the next best option to prevent pressure on the surgical site and promote drainage is on the left side, allowing the right side to remain elevated or free from direct pressure. The prone position is also used for the procedure itself.

65. A preparing to remove an indwelling urinary catheter. Which step should be performed immediately before gentle catheter removal?

a. Slowly inflate the balloon to check for patency

b. Ask the client to bear down while pulling the catheter

c. Deflate the balloon fully using a sterile syringe

d. Cut the tubing to release the balloon

Answer: c. Deflate the balloon fully using a sterile syringe

Explanation:

Before removing an indwelling urinary catheter, the balloon must be completely deflated to prevent trauma to the urethra during removal. This is done using a sterile syringe to withdraw all the sterile water from the inflation port.

66. Which of the following client statements indicates correct understanding of catheter care at home?

- a. "If I notice that the catheter tubing has a loop that lies above my leg. I should ask the nurse to reposition it."
- b. "It's okay to put the bag on the bedside table next to the bed."
- c. I should skip cleaning the catheter site when I shower, since it's already clean."
- d. "I can disconnect the drainage bag when I am out of bed so it's easier to move around."

Answer: a. "If I notice that the catheter tubing has a loop that lies above my leg, I should ask the nurse to reposition it."

Explanation:

To ensure proper drainage and prevent urinary tract infections, the catheter tubing and drainage bag must be kept below the level of the bladder and free of kinks or loops that could obstruct urine flow. Placing the bag on a bedside table (option b) is too high. Skipping cleaning (option c) increases infection risk. Disconnecting the bag (option d) is a major infection risk and should never be done without medical advice.

67. The nurse is positioning a client with right hemiplegia. To prevent subluxation of the client's right shoulder, the nurse should:

A Use a pillow to support the client's hand when she is sitting in a chair

b. Use a pillow to support the client's arm when she is sitting in a chair

d. Place a pillow under the axilla to elevate the elbow when she is lying in bed

e. Elevate the arm and hand above chest level when she is lying in bed

Answer: b. Use a pillow to support the client's arm when she is sitting in a chair

Explanation:

Supporting the affected arm (the right arm in this case) with a pillow when the client with hemiplegia is sitting helps maintain proper alignment of the shoulder joint and prevents subluxation (partial dislocation) caused by the weight of the unsupported limb and lack of muscle control.